

Our Ref: (T) RF/JK/ [MED-ID]

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Gastroenterology

CG89, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME]

[ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnosis: Ileocolonic Crohn's disease diagnosed in 2016.

Previous Medications:

6-Mercaptopurine

Azathioprine intolerant

Infliximab every six weeks from diagnosis, but stopped in May 2018 due to patient [MED-ID]

Restarted in November 2018, but only had two doses then stopped.

Follow-up:

Colonoscopy

Bloods and calprotectin

I reviewed Amy in the clinic this evening. [NAME] attended Victoria House in November for re her Infliximab having discontinued this the previous May. [NAME] had the first two loading doses but had no further doses. [NAME] tells me that [NAME] missed one appointment and then there was some confusion over the rescheduled appointment and as such the third of the loading dose has therefore not been given. [NAME] has therefore not had any treatment for her Crohn's since 19 th November 2018. In terms of symptoms [NAME] is getting some blood with her stool. [NAME] gets some abdominal pain on some days her bowels open once a day. On another days there are open up to four times. [NAME] says that if [NAME] gets hot, her joints hurt and this is mainly the left knee and both of her elbows. [NAME] says her weight is stable and [NAME] is not troubled by any fatigue.

Moving forward I am repeating her bloods today in conjunction with calprotectin . I think we need to reassess where we are so I have arranged for an urgent colonoscopy. If [NAME] has ongoing inflammation at this time then clearly [NAME] will need to go back on to a biologic and it may be that [NAME] was previously exposed to Infliximab twice, but with interrupted intervals that [NAME] is better served with an alternative anti-TNF such as Adalimumab. We will review this at the time of endoscopy.

With best wishes

Yours sincerely

[NAME]

[NAME]

[DICTATION-META]

[PROVIDER-NAME]

Consultant Gastroenterologist